



PROCEDURE	
Outcome Measures Completion: AMHCC and NOCC	
Scope (Staff):	All Mental Health Staff
Scope (Area):	All Mental Health Areas

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1. Aim

Mental health clinical outcome measures have been introduced at a national level into mental health services to support, develop and monitor the quality and consistency of clinical care. More recently the outcome measures have been expanded and further developed to support the implementation of activity-based funding for mental health services in Western Australia after July 2018. It is planned that this data will be used to allocate funding and resources and assist with service planning and quality improvement.

NMHS Mental Health (MH) staff who are working in inpatient and ambulatory (community) services are required, within specified timeframes, to complete Australian Mental Health Care Classification (AMHCC) measures for Activity Based Funding and to complete National

Outcome Casemix Collection (NOCC) under the Australian Mental Health Outcomes and Classification Network requirements.

Included in the AMHCC is a requirement to complete both a prospective Mental Health Phase of Care and standardised rating instruments (outcome measures) at key points in the consumer's clinical pathway. In some instances the same measures will be collected for both the AMHCC and NOCC protocols. These items are required to be entered into the Psychiatric Services Online Information System (PSOLIS) against the consumer's record. PSOLIS will assist the clinician by prompting them to enter the required measures at each relevant point in the clinical pathway.

NMHS Mental Health Services will ensure timely compliance with collection of the measures and their entry onto PSOLIS and monitor the quality of the data to ensure it accurately reflects the consumer's clinical presentation and their individualised and agreed treatment plan. Each mental health service manager is responsible for ensuring there are protocols in place to monitor, repair and report on data compliance, and to ensure the integrity of the outcome measures reported for their service.

2. NMHS MH Procedures

- The NMHS MH procedures supporting the operationalising of this policy in NMHS MH are outlined in the NMHS MH AMHCC and NOCC Business Rules 2017 which has been developed to be in alignment with the Mental Health Information Data (MiND) Collections: Data requirements for Health Service Providers document (2017).
- PSOLIS procedures relevant to AMHCC and NOCC are outlined in the following document: Department of Health WA: Psychiatric Services On-line Information Services (PSOLIS) Australian Mental Health Care Classification (AMHCC) Business Operating rules V1.0 31 October 2016. Perth; Department of Health

3. NMHS MH Local Service Procedures

3.1 Background

Mental Health Services (MHS) via their Leadership Committees are required to develop local service procedures that will meet the required standards to ensure that the rating and reporting of NOCC and AMHCC measures required to inform funding in the ABF environment are valid and reliable across services, teams and clinicians in NMHS. Each MHS must endorse and publish a document outlining their local procedures.

Process map templates (for either the admitted or non-admitted treatment settings) have been designed (**Appendix 1 and 2**) to support the service managers in this endeavour by identifying the main high level trigger points and the essential clinical and administrative actions required at each trigger point to ensure that the MHS collects and reports the outcome measures and Phase of Care required to align with the AMHCC and NOCC protocols.

Mental Health Services managers are required to either populate this template with the details of which position is responsible for each action, or generate a procedure in an alternate format that meets the standards embedded in the template and outlined below for case management.



3.2 Scope

The following NMHS MH services are out of scope for the first phase of the ABF implementation, pending advice from the National ABF team and the Mental Health Commission. All services are required to develop procedures for the purposes of recording outcome measures and required data elements.

3.3 Case Management

3.3.1 Standards

A mental health professional responsible for coordinating clinical care will be identified and made known to consumers ([NSQHSS User Guide for Acute and Community Mental Health Services](#)).

All patients and consumers will be allocated a Case Manager from entrance to exit of the service ([NMHS MH Case Management Procedure](#)).

3.3.2 Identifying and allocating the Case Manager

The Case Manager must be a clinician who is able to follow the consumer through the episode of care in the current treatment setting. A consumer who is active in an admitted setting cannot have a designated Case Manager who is not working in that admitted setting for the purposes of collecting and reporting AMHCC and NOCC data.

3.3.3 Admitted settings

In the acute in-patient settings (including HITH and MHOA) the Case Manager will be an appropriately skilled and qualified clinician from the multi-disciplinary team.

3.3.4 Non-Admitted Settings

In the ambulatory settings and the Hospital Extended Care Service Graylands the Case Manager will be an appropriately skilled and qualified clinician from the multi-disciplinary team.

3.3.5 Delegation of Case management tasks

The Case manager cannot delegate the responsibilities of a Case Manager however they may delegate a task related to case management to another staff member. If the Case manager delegates a task to another staff member they are responsible for ensuring that the delegate has the appropriate skills and is competent to carry out the task, and for providing oversight and supervision to the delegate as required. The tasks of rating the Phase of Care (PoC) and the NOCC outcome measures can only be delegated to a staff member who has had recent training in PoC and NOCC.

Clinical handover standards and procedures apply when there is a change of Case Manager during an episode of care.

4. PSOLIS data entry

If local Mental Health Services do not require clinicians to enter the AMHCC and NOCC ratings, and management plans, directly into PSOLIS (and print copies for the medical record) then a paper based system will need to be implemented to allow other staff to enter this information into PSOLIS (and print copies for the medical record) on their behalf in a timely fashion.



5. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

6. Compliance and Evaluation

Mental Health Services demonstrate 95% compliance with Phase of Care and Outcome measures according to the AMHCC and NOCC protocol requirements.

Related internal policies, procedures and guidelines

[DoH Patient Activity Data Procedure](#)

[NMHS MH Case Management Procedure](#)

References

[NSQHSS User Guide for Acute and Community Mental Health Services](#)



Policy Sponsor	Director of Nursing MHPHDS				
Policy Contact	Director of Nursing MHPHDS				
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Approved by:	Mental Health Policy Coordination Committee			Date:	
Endorsed by:	Director of Nursing MHPHDS			Date:	06/10/2023
NSQHS Standards Applicable:	☒ Std 1: Clinical Governance ☐ Std 2: Partnering with Consumers ☐ Std 3: Preventing and Controlling Healthcare Associated Infection ☐ Std 4: Medication Safety ☐ Std 5: Comprehensive Care ☐ Std 6: Communicating for Safety ☐ Std 7: Blood Management ☐ Std 8: Recognising and Responding to Acute Deterioration				
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Seclusion and Bodily Restraint Reduction, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					
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Appendix 1: AMHCC and NOCC Measures Process Map Template - Acute Inpatient Setting

Appendix 1

NMHS MH: AMHCC and NOCC measures process map template – Acute In-patient setting (assumes a formal clinical review occurs at least every week)						
Name of in-patient unit: 4.				Case management role: ☐ CNS ☐ Consultant Psychiatrist 1.		
Trigger point:	Admission	Prepare for 1st clinical review	1 st clinical review	Weekly clinical review:		Discharge
				PoC no change	Change to PoC	
Clinical action	Identify the case manager for the consumer	Rate and draft: PoC Outcome measures Management plan Risk assessment	Endorse: PoC Outcome measures Management plan Risk assessment	Review: Outcome measures Management plan Risk assessment	Repeat or review: PoC Outcome measures Management plan Risk assessment	Rate: Outcome measures
Who is responsible for action?	_____. 2. e.g. team coordinator, team leader or CNS	Case manager *or if delegated: _____	Present all items: Case manager or * if delegated: _____ Endorse all items: Consultant or delegate	Present all items: Case manager *or if delegated: _____ Endorse all items: Consultant or delegate	Present all items: Case manager *or if delegated: _____ Endorse all items: Consultant or delegate	Case manager *or if delegated: _____
Clinical documentation	SSCD protocols	Generate drafts	File final copies of all measures and plans on medical record	File final copies of all measures and plans on medical record	File final copies of all measures and plans on medical record	Complete discharge documentation NOCC ratings and file
Who is responsible for clinical documentation?	Clinicians as per service procedures	Case manager *or if delegated: _____	Case manager *or if delegated: _____	Case manager *or if delegated: _____	Case manager *or if delegated: _____	Case manager *or if delegated: _____
PSOLIS action	Enter case manager name on PSOLIS	Enter into PSOLIS	Confirm or update on PSOLIS	update on PSOLIS if necessary	Confirm or update on PSOLIS	Enter NOCC into PSOLIS
Who is responsible for PSOLIS actions?	Admin on advice from 2. _____	Case manager *or if delegated: _____	Case manager *or if delegated: _____	Case manager to update if necessary *or if delegated: _____	Case manager to update if necessary *or if delegated: _____	Case manager *or if delegated: _____
Instructions for completing this process map for your service	1. tick title of the position that can take the role of the case manager 2. enter the title of the position which is ultimately responsible for identifying each consumer's case manager 4. enter the name of the in-patient unit and retain formatting of row * enter the title of the position to which this case management task can be routinely delegated in this setting (remove the word 'if' and the underline, and leave 'case manager' text to indicate this is the case manager's responsibility) If PSOLIS actions are delegated, a paper based procedure will need to be implemented					

Appendix 2: AMHH and NOCC Measures Process Map Template – Ambulatory settings / or non-acute in patient settings (HECS)

Appendix 2

NMHS MH : AMHCC and NOCC measures process map template - Ambulatory settings / or non- acute in-patient setting (HECS)							
Name of Service or Program: 4.					(assuming a formal clinical review occurs at least every 3 months)		
Trigger point:	Activation or Admission	Prepare for 1 st clinical review	1 st clinical review	3 monthly clinical review:	Change to PoC before 3 monthly review is due will trigger an Ad Hoc clinical review:		Discharge/de-activation
				No change to PoC	Prepare for team meeting	Ad hoc MDT clinical review	
Clinical action	Identify the case manager for the consumer	Rate and draft: PoC Outcome measures Management plan Risk assessment	Endorse: PoC Outcome measures Management plan Risk assessment	Review: Outcome measures Management plan Risk assessment	Rate and draft: PoC Outcome measures Management plan Risk assessment	Review: PoC Outcome measures Management plan Risk assessment	Rate: Outcome measures
Who is responsible for action?	_____ 2. e.g. team coordinator, team leader or CNS	Case manager	Present all items: Case manager Endorse all items: Consultant or delegate	Present all items: Case manager Endorse all items: Consultant or delegate	Case manager	Present all items: Case manager Endorse all items: Consultant or delegate	Case manager
Clinical documentation	SSCD protocols	Generate drafts	File final copies of all measures and plans on medical record	File final copies of all measures and plans on medical record	File final copies of all measures and plans on medical record	File final copies of all measures and plans on medical record	Complete discharge documentation NOCC ratings and file
Who is responsible for clinical documentation?	Case manager	Case manager	Case manager	Case manager	Case manager	Case manager	Case manager
PSOLIS action	Enter case manager name on PSOLIS	Enter into PSOLIS	Confirm or update on PSOLIS	Confirm or update on PSOLIS	Enter on PSOLIS	Confirm or update on PSOLIS Set next 3/12 review date	Enter NOCC into PSOLIS
*Who is responsible for PSOLIS actions?	Admin on advice from _____ 2.	Case manager	Case manager	Case manager	Case manager	Case manager	Case manager
Instructions for completing this process map for your service	2. enter the title of the position which is ultimately responsible for identifying each consumer's case manager 4. enter the name of the service, program (or HECS service) and retain formatting of row *If PSOLIS actions are delegated, a paper based procedure will need to be implemented						